

Restraints

Guideline Title: Restraints

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CCCXVI. **PURPOSE:** To ensure the safety of the transport team and enhance the rendering of appropriate care when there is the potential for patient combativeness. Since a combative patient in flight can pose a serious safety threat to themselves and to the transport team, it is important to perform a thorough assessment prior to utilization of restraints. This end goal may be met utilizing either chemical or mechanical methods, as well as both concurrently. It is also realized that this document is a guideline for incremental restraint, which the provider's clinical judgment may alter to meet the goals of restraint quickly, for the safety of the patient and provider.

CCCXVII. **DEFINITIONS:**

CCCXVIII. **DEPARTMENT GUIDELINE:**

A. EQUIPMENT:

- i. Soft restraints for wrists and ankles
- ii. Sedative medications
- iii. Monitoring devices: End-tidal Carbon dioxide (ETCO₂), ECG, pulse oximetry, and BP

B. INDICATIONS:

- i. **Combative behavior** in which the patient poses a safety threat to themselves or to the medical transport team. Predictors of violent or combative behavior are: history of violence, behavior indicating potential violence through posture, speech, or motor activity, alcohol or drug intoxication, alteration of consciousness, psychiatric disorder with acute process or behaviors occurring, exposure to chemicals or smoke, or altered level of consciousness associated with altering environmental factors, or central nervous system trauma with altered level of consciousness with actual combative behavior.
- ii. Non-violent restraints to limit patient mobility, avoid treatment interruptions, or to enable active staff interventions safely.

C. **CONTRAINDICATIONS:** Ability to ensure the patient's safety with less restrictive measures

D. **PROCEDURE:**

- i. Assess patient for a physiologic etiology for uncooperative behavior such as hypoglycemia, hypoxia, and hypoperfusion. Manage as appropriate.
- ii. Attempt de-escalation techniques and use therapeutic communication.
- iii. Complete a neurologic examination and measure vital signs. Vital signs and neurologic status must be continually monitored while a patient remains restrained in any fashion.
- iv. Soft wrist and ankle restraints can be utilized as a non-violent restraint. Quick release methods are to be used to secure according to manufactures instructions.
- v. Restraint devices from sending facility may be continued such as anchored mitts.
- vi. Consideration of sedation vs. physical restraint with the following options:
 1. Chemical Sedation per *GUID-G006 – Anxiety/Combative Patient Management*
 2. Physical restraint:
 - a. Soft restraints when used should be applied to the wrist and ankles at a comfortable angle
 - b. Restraints should be anchored to either the stretcher or the backboard to allow the patient to be placed on their side in the event of emesis.
 - c. When applying soft restraints, allow for enough room underneath the device so that one finger maybe inserted between the restraint and the extremity.
 - d. Circulation, sensory and motor function of all restrained patients should be assessed every 10 minutes to coincide with assessment and documentation of vital signs.
- vii. The patient in custody shall be placed in 4-point restraints or handcuffed (feet and hands) by law enforcement agents. This is considered a forensic restraint. The team shall be responsible for assessing distal circulation. The police/correction officer should accompany the patient during the transport, with approval of the pilot in command.
- viii. There are times when ground transportation is most appropriate for patients. These situations may include situations such as combative patients. The general principle is that the team should go with the patient even if this requires going by ground. If the medical transport team determines that transporting the patient is considered unsafe due to combativeness, the team may refuse to transport the patient by air and accompany the patient via ground ambulance.

- ix. If sedation and physical restraints have been ineffective or in the provider's sound medical judgment the clinical situation requires chemical restraint with paralyzation, this may be utilized per *GUID-G008 – Rapid Sequence induction for Endotracheal Intubation and GUID-PR020 Endotracheal Intubation*.
- x. Carefully document the medical decision-making that led the medical transport team to choose such a method of restraint.

CCCXIX. **DOCUMENTATION:** EMS Charts

CCCXX. **REFERENCES:**

- A. Orlando Health Policy #2580 Restraint Use in the Acute Care Setting